

IN THE HOUSE OF REPRESENTATIVES

HOUSE BILL NO. 841

BY WAYS AND MEANS COMMITTEE

AN ACT

RELATING TO HEALTH INSURANCE; AMENDING TITLE 41, IDAHO CODE, BY THE ADDITION OF A NEW CHAPTER 35, TITLE 41, IDAHO CODE, TO ESTABLISH THE IDAHO PRIOR AUTHORIZATION REFORM ACT, TO PROVIDE A SHORT TITLE, TO PROVIDE THE PURPOSE OF THE CHAPTER, TO PROVIDE FOR APPLICABILITY AND SCOPE, TO DEFINE TERMS, TO PROVIDE FOR DISCLOSURE AND REVIEW OF PRIOR AUTHORIZATION REQUIREMENTS, TO PROVIDE FOR PRIOR AUTHORIZATION APPLICATION PROGRAMMING INTERFACE, TO PROVIDE FOR STANDARD PRIOR AUTHORIZATIONS, TO PROVIDE FOR EXPEDITED PRIOR AUTHORIZATIONS, TO PROVIDE FOR NOTIFICATIONS FOR ADVERSE DETERMINATIONS, TO PROVIDE FOR PERSONNEL QUALIFIED TO REVIEW APPEALS, TO PROVIDE FOR INSURER REVIEW OF PRIOR AUTHORIZATION REQUIREMENTS, TO PROVIDE FOR REVOCATION OF PRIOR AUTHORIZATIONS, TO PROVIDE FOR THE LENGTH OF APPROVALS, TO PROVIDE FOR APPROVALS FOR CHRONIC CONDITIONS, TO PROVIDE FOR CONTINUITY OF PRIOR APPROVALS, TO PROVIDE FOR ENFORCEMENT AND ADMINISTRATION, TO PROVIDE FOR REPORTS TO THE DEPARTMENT OF INSURANCE, TO PROVIDE FOR FALSE REQUESTS FOR PRIOR AUTHORIZATION, TO PROVIDE FOR A DE MINIMIS PRIOR AUTHORIZATION UTILIZATION EXEMPTION, AND TO PROVIDE RULEMAKING AUTHORITY; AND PROVIDING AN EFFECTIVE DATE.

Be It Enacted by the Legislature of the State of Idaho:

SECTION 1. That Title 41, Idaho Code, be, and the same is hereby amended by the addition thereto of a NEW CHAPTER, to be known and designated as Chapter 35, Title 41, Idaho Code, and to read as follows:

CHAPTER 35

IDAHO PRIOR AUTHORIZATION REFORM

41-3501. SHORT TITLE. This chapter shall be known and may be cited as the "Idaho Prior Authorization Reform Act."

41-3502. PURPOSE. The purpose of this chapter is to:

- (1) Protect the health care provider-patient relationship from unreasonable third-party interference;
- (2) Prevent prior authorization programs from hindering the independent medical judgment of a physician or other health care provider; and
- (3) Ensure the transparency of a fair and consistent process for health care providers and their patients.

41-3503. APPLICABILITY AND SCOPE. This chapter applies to every health benefit plan, as defined in section 41-5903, Idaho Code, to all issuers of health benefit plans, to all incorporated or stand-alone dental benefit plans, and to all utilization reviews and utilization review organizations, as defined in section 41-5903, Idaho Code, except for employee or employer self-insured health benefit plans under the federal employee

retirement income security act of 1974, health care provided pursuant to worker's compensation law, or prescription drugs, biologics, biosimilars, and pharmaceutical medicines. This chapter does not diminish the duties and responsibilities under other federal or state law or rules promulgated under those laws applicable to a health insurer, health insurance issuer, health benefit plan, utilization review plan, or utilization review organization.

41-3504. DEFINITIONS. For the purposes of this chapter:

(1) "Adverse determination" means a determination by a health insurance issuer that, based on the information provided, a pre-service request for a benefit under the health insurance issuer's health benefit plan upon application of any utilization review technique does not meet the health insurance issuer's requirements for medical necessity, appropriateness, health care setting, level of care, or effectiveness or is determined to be experimental or investigational, and the requested benefit is therefore denied.

(2) "Appeal" means a formal request, either orally or in writing, to reconsider an adverse determination.

(3) "Approval" means a determination by a health insurance issuer that a health care service has been reviewed and, based on the information provided, satisfies the health insurance issuer's requirements for medical necessity and appropriateness.

(4) "Clinical review criteria" means the written screening procedures, decision abstracts, clinical protocols, and practice guidelines used by a health insurance issuer to determine the necessity and appropriateness of health care services.

(5) "Complete prior authorization request" means a prior authorization request that:

(a) Is submitted by a health care professional or health care provider in accordance with the standardized electronic prior authorization process required by this chapter, if applicable;

(b) Includes all clinical documentation, diagnostic results, and other information reasonably required by the health insurance issuer's publicly disclosed clinical review criteria in effect at the time the request is submitted;

(c) Requires no additional information from the enrollee, the provider, or a third party that is reasonably necessary to adjudicate the request; and

(d) Complies with the health insurance issuer's published prior authorization submission standards in effect at the time the request is submitted.

(6) "Dentist" means any person with a valid doctor of dental surgery, doctor of medicine in dentistry, or doctor of dental medicine degree.

(7) "Department" means the Idaho department of insurance.

(8) "Emergency medical condition" means a medical condition manifesting itself by acute symptoms of sufficient severity, including but not limited to severe pain, such that a prudent layperson who possesses an average knowledge of health and medicine could reasonably expect the absence of immediate medical attention to result in:

- 1 (a) Placing the health of the individual or, with respect to a pregnant
2 woman, the health of the woman or her unborn child, in serious jeopardy;
3 (b) Serious impairment to bodily functions; or
4 (c) Serious dysfunction of any bodily organ or part.
- 5 (9) "Emergency services" means health care items and services fur-
6 nished or required to evaluate and treat an emergency medical condition.
- 7 (10) "Enrollee" means any person and the person's dependents enrolled
8 in or covered by a health care plan.
- 9 (11) (a) "Expedited prior authorization request" means a pre-service or
10 concurrent care claim for medical care or treatment for which applica-
11 tion of the time periods for making a non-expedited prior authorization
12 could, in the opinion of a treating health care professional or health
13 care provider with knowledge of the enrollee's medical condition:
- 14 (i) Seriously jeopardize the life or health of the enrollee or the
15 ability of the enrollee to regain maximum function;
16 (ii) Subject the enrollee to severe pain that cannot be adequately
17 managed without the care or treatment that is the subject of the
18 authorization request; or
19 (iii) Lead to likely onset of an emergency medical condition if the
20 service is not rendered during the time period to render a prior
21 authorization determination for an urgent medical service.
- 22 (b) "Expedited prior authorization request" does not apply to emer-
23 gency services.
- 24 (12) "Health care professional" means a physician, a registered pro-
25 fessional nurse, a dentist, or another individual appropriately licensed or
26 registered to provide health care services.
- 27 (13) "Health care provider" means any physician, dentist, hospital, am-
28 bulatory surgery center, or other person or facility that is licensed or oth-
29 erwise authorized to deliver health care services.
- 30 (14) "Health care service" means any services or level of services in-
31 cluded in the furnishing of medical or dental care to an individual or the
32 hospitalization incident to the furnishing of such care, as well as the fur-
33 nishing of any other services to any person for the purpose of preventing,
34 alleviating, curing, or healing human illness or injury, including behav-
35 ioral health, mental health, and home health, and pharmaceutical services,
36 products, and medications.
- 37 (15) "Health insurance issuer" means the issuer of a health benefit plan
38 or dental benefit plan.
- 39 (16) "Medically necessary" means care that a health care professional
40 exercising prudent clinical judgment would provide to a patient for the pur-
41 pose of preventing, diagnosing, or treating an illness, injury, disease, or
42 its symptoms and that is:
- 43 (a) In accordance with generally accepted standards of medical prac-
44 tice or dental practice;
45 (b) Clinically appropriate in terms of type, frequency, extent, site,
46 and duration and considered effective for the patient's illness, in-
47 jury, or disease;
48 (c) Focused on what is best for the patient's health outcome; and

(d) Not primarily for the convenience of the patient, treating physician, other health care professional, caregiver, family member, or other interested party.

(17) "Physician" means any person with a valid doctor of medicine, doctor of osteopathy, or doctor of podiatry degree.

(18) "Prior authorization" means the process by which a health insurance issuer determines the medical necessity and medical appropriateness of an otherwise covered health care service before the rendering of such health care service. While not requiring explicit approval, any notification required of an enrollee, health care professional, or health care provider by the health insurance issuer before, at the time of, or concurrent to providing a health care service shall be included within the definition of "prior authorization." Any pre-service review that is used to evaluate clinical necessity, regardless of the terminology used to describe such pre-service review, falls within this definition, including but not limited to predetermination, pretreatment, and preauthorization.

(19) "Urgent health care services" means those services that, if not provided to an enrollee, could seriously jeopardize the enrollee's life, health, or ability to regain maximum function.

(20) "Utilization review organization" has the meaning given to that term in section 41-5903, Idaho Code.

41-3505. DISCLOSURE AND REVIEW OF PRIOR AUTHORIZATION REQUIREMENTS. (1) A health insurance issuer shall maintain a complete list of services for which prior authorization is required, including for all services where prior authorization is performed by an entity under contract with the health insurance issuer.

(2) A health insurance issuer shall make any current prior authorization requirements and restrictions, including the written clinical review criteria, readily accessible and conspicuously posted on its website or online portal to enrollees, health care professionals, and health care providers. Content published by a third party and licensed for use by a health insurance issuer may be made available through the health insurance issuer's secure, password-protected website or online portal as long as the access requirements of the website do not unreasonably restrict access. Requirements shall be described in detail, written in easily understandable language, and readily available to the health care professional and health care provider at the point of care. The website or online portal shall indicate for each service subject to prior authorization:

(a) The date on which prior authorization became required for policies issued or health benefit plan documents delivered in Idaho, including the effective dates and the termination dates, if applicable, in Idaho;

(b) The date on which the Idaho-specific requirement was listed on the website or online portal of the health insurance issuer;

(c) If applicable, the date on which the prior authorization requirement was removed for Idaho; and

(d) If applicable, access to a standardized electronic prior authorization request transaction process.

(3) The clinical review criteria must:

1 (a) Be consistent with nationally accepted standards generally recog-
2 nized by physicians and health care providers practicing in relevant
3 medical and clinical specialties except where state law provides its
4 own standard;

5 (b) Be developed in accordance with the current standards of a national
6 medical accreditation entity;

7 (c) Ensure quality of care and access to needed health care services;

8 (d) Be based on evidence from sources, including peer-reviewed scien-
9 tific studies;

10 (e) Be sufficiently flexible to allow deviations from norms when justi-
11 fied on a case-by-case basis; and

12 (f) Be evaluated and updated at least annually under the direction of a
13 physician who:

14 (i) Possesses a current, valid, and unrestricted license to prac-
15 tice medicine in Idaho or in a state with substantially similar li-
16 censing requirements; and

17 (ii) Has knowledge of the standard of care in the community where
18 care is proposed to be provided.

19 (4) A health insurance issuer shall not deny a claim for failure to ob-
20 tain prior authorization if the prior authorization requirement was not in
21 effect on the date of service or if the claim or prior authorization require-
22 ments were not publicly disclosed by the plan on the health insurance is-
23 suer's website, online portal, or other materials.

24 (5) If a health insurance issuer intends either to implement a new prior
25 authorization requirement or restriction or to amend an existing require-
26 ment or restriction, the health insurance issuer shall provide impacted
27 enrollees, contracted health care professionals, and contracted health care
28 providers of enrollees written notice of the new or amended requirement no
29 less than sixty (60) days before the requirement or restriction is imple-
30 mented. Written notice may take the form of a conspicuous notice posted
31 on the health insurance issuer's public website or online portal for con-
32 tracted health care professionals and contracted health care providers or
33 email notice to health care professionals or health care providers. A health
34 insurance issuer shall provide email notices to all impacted enrollees and
35 to health care professionals or health care providers if the health care
36 professional or health care provider has requested to receive the notice
37 through email. A new or amended requirement shall not be implemented unless
38 the health insurance issuer's website or online portal has been updated to
39 reflect the new or amended requirement or restriction. Written notice of a
40 new, amended, or restricted prior authorization requirement may be provided
41 less than sixty (60) days in advance of implementation if a health insurance
42 issuer determines and contemporaneously notifies the department in writing
43 that:

44 (a) The health insurance issuer has identified fraudulent or abusive
45 practices related to the health care service;

46 (b) The health care service is unavailable or scarce, necessitating the
47 use of an alternative health care service;

48 (c) The health care service is newly introduced to the health care mar-
49 ket and a delay in providing coverage for the health care service would
50 not be in the best interests of enrollees;

(d) The health care service is the subject of a clinical trial authorized by the United States food and drug administration;

(e) Changes to the health care service or its availability are otherwise required by law to be made by the health insurance issuer in less than sixty (60) days; or

(f) The prior authorization requirement is being removed.

(6) Health insurance issuers using prior authorization shall make statistics available regarding prior authorization approvals and denials on their website or online portal in a readily accessible format. The reporting requirements of this subsection shall be implemented in a manner consistent with the public reporting requirements established under the centers for medicare and medicaid services (CMS) interoperability and prior authorization final rule (CMS-0057-F), as amended, to the extent applicable.

(7) The implementation requirements of this section shall commence by January 1, 2027. Health insurance issuers may comply with the implementation requirements of this section in phases, consistent with applicable federal interoperability timelines.

41-3506. PRIOR AUTHORIZATION APPLICATION PROGRAMMING INTERFACE. (1) If a health insurance issuer requires prior authorization of a health care service, the issuer or its contracted utilization review organization shall, to the extent required under applicable federal interoperability requirements, and by January 1, 2027, implement and maintain a prior authorization application programming interface (API).

(2) The API shall:

(a) Conform to the applicable interoperability standards adopted by the centers for medicare and medicaid services (CMS) in the CMS interoperability and prior authorization final rule (CMS-0057-F), including health level 7 fast healthcare interoperability resources release 4.0.1 or a successor version adopted by CMS;

(b) Be capable of identifying, for items and services excluding prescription drugs unless otherwise required by federal law, whether prior authorization is required;

(c) Identify payer-specific documentation requirements for each item or service that requires prior authorization;

(d) Support the electronic creation and exchange of prior authorization requests and responses between health care professionals, health care providers, and the health insurance issuer; and

(e) Be implemented in a manner that does not disrupt compliance with federal interoperability requirements.

(3) A health insurance issuer may use an updated version of a required interoperability standard if such use is consistent with federal law and does not disrupt an end user's ability to access required data.

(4) Nothing in this section shall require the creation of an Idaho-specific electronic prior authorization.

(5) For the purposes of this chapter, a prior authorization request shall be deemed received only upon submission of a complete prior authorization request.

(a) A request shall be considered complete when the health insurance issuer has received all documentation and information reasonably nec-

1 essary to make a determination under the terms of the health benefit
2 plan.

3 (b) A prior authorization request shall not fail to meet the definition
4 of a complete prior authorization request due to minor clerical or tech-
5 nical errors that do not materially affect the ability of the health in-
6 surance issuer to make a determination.

7 (c) A prior authorization request shall be presumed complete unless the
8 health insurance issuer provides written notice within one (1) business
9 day of receipt specifying with particularity any additional informa-
10 tion reasonably necessary to adjudicate the request.

11 41-3507. STANDARD PRIOR AUTHORIZATIONS. (1) If a health insurance
12 issuer requires prior authorization of a health care service, the health
13 insurance issuer shall render a decision and notify the enrollee and the en-
14 rollee's health care professional or health care provider as expeditiously
15 as the enrollee's condition requires but no later than seven (7) calen-
16 dar days after receipt of a complete prior authorization request, unless a
17 longer time frame is required under applicable federal law. Requests for
18 additional information must be reasonably necessary to adjudicate the prior
19 authorization request. As used in this section, "additional information"
20 may include the results of any face-to-face clinical evaluation, a second
21 opinion, or any other clinical information that is directly applicable to
22 the requested service as may be required.

23 (2) If a health insurance issuer determines that a prior authorization
24 request is incomplete, the health insurance issuer shall notify the health
25 care professional or health care provider within one (1) business day and
26 shall specify in writing the information reasonably necessary to complete
27 the request. The health insurance issuer may request additional information
28 only once per prior authorization request unless the health care provider
29 submits materially new clinical information.

30 41-3508. EXPEDITED PRIOR AUTHORIZATIONS. (1) If requested by a treat-
31 ing health care professional or health care provider for an enrollee, a
32 health insurance issuer shall render a decision concerning urgent health
33 care services as expeditiously as the enrollee's condition requires, but
34 no later than seventy-two (72) hours after receipt of a complete expedited
35 prior authorization request, unless a longer time frame is required pursuant
36 to applicable federal law.

37 (2) To facilitate the rendering of a prior authorization determina-
38 tion pursuant to this section, a health insurance issuer shall establish a
39 mechanism to ensure health care professionals have access to appropriately
40 trained and licensed physicians preferably but not necessarily of the same
41 specialty for consultation, designated by the health insurance issuer to
42 make such determinations for prior authorization concerning urgent care
43 services.

44 41-3509. NOTIFICATIONS FOR ADVERSE DETERMINATIONS. If a health in-
45 surance issuer makes an adverse determination, the health insurance issuer
46 shall include the following in the notification to the enrollee and the en-
47 rollee's health care professional or health care provider:

1 (1) The reasons for the adverse determination and related evi-
 2 dence-based criteria, including a description of any missing or insuffi-
 3 cient documentation;
 4 (2) The right to appeal the adverse determination;
 5 (3) Instructions on how to file the appeal;
 6 (4) Additional documentation necessary to support the appeal; and
 7 (5) The right to request an independent external review pursuant to the
 8 provisions of chapter 59, title 41, Idaho Code.

9 41-3510. PERSONNEL QUALIFIED TO REVIEW APPEALS. A health insurance
 10 issuer shall ensure that all appeals are peer reviewed by an appropriate
 11 licensed health care professional in the same or substantially similar field
 12 or specialty. The reviewing health care professional shall:

13 (1) Possess a current and valid nonrestricted license to practice
 14 medicine with substantially similar licensing requirements to this state;
 15 (2) Be certified by the American board of medical specialties or the
 16 American osteopathic association within the relevant specialty of a physi-
 17 cian who typically manages the medical condition or disease;
 18 (3) Have training, knowledge, or experience of providing the health
 19 care services under appeal;
 20 (4) Not have been directly involved in making the adverse determina-
 21 tion; and
 22 (5) Consider all known clinical aspects of the health care service un-
 23 der review, including a review of all pertinent medical records provided to
 24 the health insurance issuer or health care provider, the health plan's clin-
 25 ical guidelines, and peer-reviewed scientific studies.

26 41-3511. INSURER REVIEW OF PRIOR AUTHORIZATION REQUIREMENTS. A health
 27 insurance issuer shall periodically review its prior authorization require-
 28 ments and consider removal of prior authorization requirements.

29 41-3512. REVOCATION OF PRIOR AUTHORIZATIONS. (1) A health insurance
 30 issuer may not revoke or further limit, condition, or restrict a previously
 31 issued prior authorization approval while it remains valid in accordance
 32 with this chapter unless:

33 (a) The health insurance issuer has identified fraudulent or abusive
 34 practices related to the health care service;
 35 (b) The health care service is unavailable, necessitating the use of an
 36 alternative health care service;
 37 (c) The health care service is the subject of a new safety alert from the
 38 United States food and drug administration or is in response to a public
 39 health emergency;
 40 (d) The change is based on nationally recognized generally accepted
 41 standards developed in accordance with current standards of a national
 42 medical accreditation entity or specialty society;
 43 (e) Changes to the health care service or its availability are other-
 44 wise required by law to be made by the health insurance issuer within
 45 sixty (60) days; or
 46 (f) There is a material change in clinical circumstances that is sup-
 47 ported by documented medical evidence.

(2) Notwithstanding any other provision of law, if a claim is properly coded and timely submitted to a health insurance issuer, the health insurance issuer shall make payment according to the terms of coverage on claims for health care services for which prior authorization was required and approval received before the provision of health care services unless:

(a) It is determined that the enrollee's health care professional or health care provider knowingly and without exercising prudent clinical judgment provided health care services that required prior authorization from the health insurance issuer or its contracted utilization review organization without first obtaining prior authorization for such health care services;

(b) It is timely determined that the health care services claimed were not performed;

(c) It is timely determined that the health care services provided by the enrollee's health care professional or health care provider were contrary to the instructions of the health insurance issuer or its contracted utilization review organization if contact was made between such parties before the health care services being provided;

(d) It is timely determined that the person receiving such health care services was not an enrollee of the health care plan; or

(e) The approval was based on a material misrepresentation by the enrollee, health care professional, or health care provider. As used in this paragraph, "material" means a fact or situation that would have resulted in a substantial change in the determination had it been accurately disclosed in the submission.

(3) Nothing in this section shall preclude a health insurance issuer or a utilization review organization from performing post-service reviews of health care claims for purposes of payment integrity or for the prevention of fraud, waste, or abuse.

41-3513. LENGTH OF APPROVALS. (1) A prior authorization approval shall be valid for six (6) months after the date the health care professional or health care provider receives the prior authorization approval. Provided, however, a health insurance issuer and an enrollee or enrollee's health care professional may extend a prior authorization approval for a longer period, by agreement.

(2) Nothing in this section shall require a policy or plan to cover any care, treatment, or services for any health condition that the terms of coverage otherwise completely exclude from the policy's or plan's covered benefits without regard for whether the care, treatment, or services are medically necessary.

41-3514. APPROVALS FOR CHRONIC CONDITIONS. (1) If a health insurance issuer requires a prior authorization for a recurring health care service for the treatment of a chronic or long-term condition, the approval shall remain valid for the lesser of twelve (12) months from the date the health care professional or health care provider receives the authorization approval or the length of the treatment as determined by the patient's health care professional. Provided, however, a health insurance issuer and an enrollee or

1 the enrollee's health care professional may extend a prior authorization ap-
2 proval for a longer period, by agreement.

3 (2) Nothing in this section shall require a policy or plan to cover any
4 care, treatment, or services for any health condition that the terms of cov-
5 erage otherwise completely exclude from the policy's or plan's covered ben-
6 efits without regard for whether the care, treatment, or services are medi-
7 cally necessary.

8 41-3515. CONTINUITY OF PRIOR APPROVALS. (1) Upon receipt of informa-
9 tion documenting a prior authorization approval from the enrollee or from
10 the enrollee's health care professional or health care provider, a health
11 insurance issuer shall honor a prior authorization granted to an enrollee
12 from a previous health insurance issuer for at least the initial ninety (90)
13 days of an enrollee's coverage under a new health plan, subject to the terms
14 of the enrollee's coverage agreement.

15 (2) During the time period described in subsection (1) of this section,
16 a health insurance issuer may perform its own review to grant a prior autho-
17 rization approval, subject to the terms of the enrollee's coverage agree-
18 ment.

19 (3) Nothing in this chapter shall require a policy or plan to cover any
20 care, treatment, or services for any health condition that the terms of cov-
21 erage otherwise completely exclude from the policy's or plan's covered ben-
22 efits without regard for whether the care, treatment, or services are medi-
23 cally necessary.

24 (4) Nothing in this chapter shall prevent a health insurance issuer to
25 engage an enrollee with an option to consider clinically appropriate alter-
26 natives.

27 41-3516. ENFORCEMENT AND ADMINISTRATION. (1) In addition to the en-
28 forcement powers granted to it by law to enforce the provisions of this chap-
29 ter, the department is granted specific authority to issue a cease-and-de-
30 sist order or require a health insurance issuer or utilization review organ-
31 ization, or both, to submit a plan of correction for violations of this chap-
32 ter. Subject to rules promulgated by the department pursuant to chapter 52,
33 title 67, Idaho Code, and after proper notice and the opportunity for a hear-
34 ing, the department may impose on a health insurance issuer, health benefit
35 plan, or utilization review organization an administrative fine not to ex-
36 ceed ten thousand dollars (\$10,000) per violation for failure to submit a re-
37 quested plan of correction, failure to comply with its plan of correction,
38 or repeated violations of this chapter. All fines collected by the depart-
39 ment pursuant to this section shall be deposited in the state general fund.
40 The department may also exercise all authority granted to it under the pro-
41 visions of chapter 59, title 41, Idaho Code, to deny or revoke approval of a
42 utilization review organization for a violation of this chapter.

43 (2) An enrollee or an enrollee's health care provider who has evidence
44 that the enrollee's health insurance issuer or health benefit plan is in
45 violation of the provisions of this chapter may file a complaint with the
46 department. The department shall review all complaints received and in-
47 vestigate all complaints that it deems to state a potential violation. The
48 department shall fairly, efficiently, and timely review and investigate

1 complaints and shall provide the subject of the complaint an opportunity to
 2 refute the evidence against it. Health insurance issuers, health benefit
 3 plans, and utilization review organizations found to be in violation of this
 4 chapter shall be penalized in accordance with this section.

5 (3) There shall be no private right of action under this chapter.

6 41-3517. REPORTS TO THE DEPARTMENT. (1) By June 1, 2027, and each June
 7 1 thereafter, a health insurance issuer shall report to the department, on a
 8 form issued by the department, the following aggregated trend data, de-iden-
 9 tified of protected health information, related to the insurer's practices
 10 and experience for the prior plan year for health care services submitted for
 11 payment:

12 (a) The number of prior authorization requests;

13 (b) The percentage of prior authorization requests denied;

14 (c) The percentage of prior authorization appeals received;

15 (d) The percentage of adverse determinations reversed on appeal;

16 (e) The percentage of prior authorization requests that were not sub-
 17 mitted electronically;

18 (f) As a percentage by service, the ten (10) health care services that
 19 were most frequently denied through prior authorization; and

20 (g) The five (5) reasons prior authorization requests were most fre-
 21 quently denied.

22 (2) All reports required by this section shall be considered public
 23 records pursuant to chapter 1, title 74, Idaho Code, and the department shall
 24 make all reports freely available to requesters and post all reports to its
 25 public website without redactions.

26 41-3518. FALSE REQUESTS FOR PRIOR AUTHORIZATION. If a health insur-
 27 ance issuer has clear and convincing evidence that a health care profes-
 28 sional or health care provider has knowingly and willfully submitted false
 29 or fraudulent requests for prior authorization to the health insurance is-
 30 suer, the health insurance issuer shall notify and provide that information
 31 to the department director. After receipt of such notification and infor-
 32 mation, the director shall forward these reports to the board of medicine or
 33 such other licensing agency with oversight of the health care professional
 34 or health care provider and to the office of the prosecuting authority having
 35 jurisdiction.

36 41-3519. DE MINIMIS PRIOR AUTHORIZATION UTILIZATION EXEMPTION. (1) A
 37 health insurance issuer that, for the prior plan year, required prior autho-
 38 rization for less than one percent (1%) of claims submitted for payment under
 39 health benefit plans issued or delivered in Idaho may elect to be exempt from
 40 the requirements of this chapter.

41 (2) The election shall be made by filing an annual attestation with the
 42 department, in a form and manner specified by the department, demonstrating
 43 that the health insurance issuer meets the threshold provided for in subsec-
 44 tion (1) of this section.

45 (3) Upon request, the health insurance issuer shall provide records
 46 reasonably necessary for the department to verify the attestation provided
 47 for in subsection (2) of this section. If the department determines the

1 health insurance issuer does not meet the threshold, the department may re-
2 voke the exemption and require compliance within a reasonable period.

3 41-3520. RULES. The department shall have the authority to promulgate
4 rules, subject to legislative approval, pursuant to the provisions of chap-
5 ter 52, title 67, Idaho Code, to govern the administration of this chapter.

6 SECTION 2. This act shall be in full force and effect on and after Jan-
7 uary 1, 2027.